

Fictional Interview and Case Study of Obsessive Compulsive Disorder

When I met Samantha, she was a well-dressed 30 year old first-year law student and financially successful bartender, working weeknights despite an extremely demanding course load. She managed her circumstantial life stressors to the best of her ability - occasionally self-medicating with alcohol but typically throwing herself full-time into work and academics as a distraction. However, the recent death of her estranged father triggered pursuit of treatment, and it became evident that her symptoms were far beyond evidence of complicated grief.

Upon our initial assessment, I inquired about her family history, personal history, and circumstances that led to seeking treatment. During the course of intake, it was evident from her body language and fidgeting that she was nervous and seemed uncomfortable being the center of attention. To start, I asked about her family. Her mother was the primary caregiver during childhood and worked excessive hours to sustain the family financially. Her father was often absent; her parents divorced soon after her birth, and he struggled with major depressive disorder and chronic substance abuse. Her parents rarely crossed paths but she witnessed extremely volatile interactions between them. An unfortunate consequence of their divorce was financial instability, including several abrupt moves between homes throughout Samantha's childhood. She recalled her mother struggled financially and emotionally, often relying on Samantha to be responsible in her absence. Her mother also had been previously diagnosed as having Generalized Anxiety Disorder but never sought treatment; she chose to self-medicate with alcohol, sometimes to an excessive degree.

As we proceeded with the initial interview about her background, I highlighted some revealing details scattered throughout the interview: she was perpetually concerned about being single, but avoided intimate relationships and made several excuses - too busy with work or school or there was a lack of "good" men. She also reported a long-term history of several distressing physical symptoms, including insomnia, high blood pressure, and chronic migraines. To manage these, she periodically abused alcohol. The specific circumstance that encouraged her to seek treatment was her father's passing; however, upon further questioning, she admitted an increased frequency of unmanageable panic attacks, disruptive insomnia, a sense of isolation despite a desire to initiate and maintain social relationships, frustration and anxiety about academic performance, periods of anhedonia, and bizarre thought cycles that seemed objectively irrational but felt very real and traumatizing.

We continued to take inventory of personal history and details; it became evident that anxiety and high expectations had been a pervasive part of her childhood. Samantha participated in a variety of challenging extracurriculars from middle school through high school, while expected to maintain high grades. While her grades were consistently above average, she struggled to maintain close friendships and experienced a series of volatile, brief relationships with men starting at age 16. She was diagnosed with Generalized Anxiety Disorder after a year of meeting with a child psychologist and psychiatrist. They prescribed an SSRI, however the high expectations placed upon her led to minor amphetamine abuse and noncompliance with anti-anxiety medication. Additionally, she spent her rare free time abusing alcohol and experimenting with drugs to cope with home and school stressors. It never occurred to her that this high-risk behavior was anything other than normal adult coping, and she continued to struggle with relationships, self-medication, and school performance through college.

After several years of struggling to develop personal and professional relationships, she felt unsuccessful relative to her peers, and decided to enroll in law school. By this point, her relationships with her mother and sister had vastly improved over time, however, her relationship with her father deteriorated to complete estrangement. It was only when he passed that her depressive and anxious symptoms became clinically significant, and it triggered a serious impairment of her ability to focus on work and school, and her symptoms caused major distress and isolation in the course of daily events.

I also noted several things as relevant to mental status during the course of intake: her appearance and behavior indicated a physical restlessness, diminutive or submissive posture, and she was a bit overdressed for the comfortable setting of a clinician's office. In the course of our first meeting, I noticed several recurring themes in her thought processes. She experienced anxiety and despair over her father's passing and indicated a broad and excessive sense of worry about her living family members. She viewed many of her current circumstances through a negative lens, hinting at a lack of satisfaction in her own performance at work and school, and refusing to acknowledge her positive grades and current financial stability.

Despite only reviewing her family and personal history, it was evident by her phrasing that she experienced a distorted sense of personal responsibility for her family member's lives and a heightened sense of worry over inconsequential social experiences and interactions. She also seemed preoccupied with her perceived appearance and school performance, concerned that others would judge her for failures or imperfections both physically and socially. None of the cognitive distortions of thought indicated hallucinations or delusions, and despite the abundance of negative affect, she did not have thoughts of suicide or self-harm.

In fact, she alternated between thought cycles of grief, intense worry, and hopefulness. Her affect, while negative, was appropriate relative to her extensive history of disruptive and traumatic familial and romantic experiences. Her intellectual functioning was also intact, and indicated a high level of intelligence and education, although she reported several periods of brain fog and forgetfulness. These periods seemed to coincide with stressful life events and inevitably led to a cycle of experiencing academic frustration followed by low feelings of self worth in regards to intellect. Her sensorium was clear and oriented times three. After our initial meeting, I recommended Samantha visit her physician to rule out any possible illness that could be contributing to her somatic symptoms. No underlying medical conditions were found in the course of her exam, and she was given a clean bill of health with the exception of pre-hypertensive blood pressure.

When we met again for consequential interviews, diagnosis, and intervention plans, I took inventory of all symptoms. Her previous diagnosis of Generalized Anxiety Disorder was insufficient, in my opinion, although she met the majority of criteria for diagnosis. Additionally, it seemed that as more was revealed, treatment would be better tailored to suit her circumstances with a more appropriate diagnosis. Current symptoms included relentless grief that exceeded a "normal" range of time, extreme fear regarding social situations and social performance, panic attacks preceding work and major school projects, physical manifestations of extreme stress such as migraines and tension, misuse of substances to self-medicate or cope with stress, cognitive distortions of thought, avoidance of social situations and relationships, intrusive and obsessive thoughts partnered with compulsive mental-review and rumination, low-self worth, anhedonia, low levels of energy and concentration, and thought-fusion action regarding several of her intrusive thoughts. The biggest example was a recurring thought she had in which she imagined her remaining living family members' deaths and began

to believe that by thinking about the occurrence, it would come to pass - just like her father, who she hated as a child, wished would go away, and was now deceased.

With all of these symptoms in mind, I initiated a diagnosis of Obsessive-Compulsive Disorder with evidence of a Major Depressive Episode. Many of her obsessive-compulsive ruminations and thought cycles seemed to originate from the unpredictability and heightened sense of responsibility she experienced during childhood, reinforced by genetic predisposition for anxiety. Her feelings of low self worth in response to feeling guilty for her father's death may have led to the grief-triggered depressive episode for which she initially sought treatment. Given her family history and background, I believe environmental factors such as parental unpredictability, home instability, financial insecurity, and impressions of substance use all contribute to high risk factors for a Major Depressive Disorder or other mood disorder, and potential substance abuse disorder under the right circumstances. Additionally, she harbored some long-term obsessive and compulsive thoughts that preceded her father's death, and felt some relief at compulsively mentally reviewing and re-reviewing her social interactions for evidence of "failing" others or harming them in some fashion. She also compulsively avoided intimate social interactions, along with discussions of death or dying, not only to avoid the possibility of a panic attack, but to relieve some of the anxiety of the intrusive thoughts about death and abandonment. She struggled with the ensuing isolation and lack of social support.

Our treatment plan was multifaceted, designed to target her existing symptoms, mitigate potential risk factors of developing any further disorders, enhance coping skills for future stressors, and improve social skills and dynamics. The main goals were not to eliminate symptoms entirely, but to mitigate the experience of anxious thoughts to allow increased functioning. Other goals were to reduce panic attacks, minimize substance use, encourage healthier habits and stress management, reframe negative cognitive distortions, reduce or eliminate compulsions, and improve physical health and affect. I started with a prescription of the tricyclic clomipramine, not only to address the symptoms of OCD, but to target depressive and anxious symptoms in general and assist with maintenance. I also recommended keeping an inventory of triggers; she was asked to self-monitor and note any circumstances preceding or surrounding a panic attack or desire to use substances.

In therapy, we started with cognitive behavioral treatment, specifically Exposure and Response Prevention, to address her recurring OCD themes of death and "harm" toward others and other CBT techniques for her extreme fear of social perception and performance. I also recommended Interpersonal Psychotherapy specifically to learn skills to manage grief and cope with the loss of a parent, learn skills to build new relationships, and identify any social deficits contributing to repeating patterns of inconsistent and rocky romantic relationships. Her social problems did not seem entirely attributable to OCD, as she met many of the criteria for Social Anxiety Disorder, so also used CBT techniques to dispel catastrophic beliefs regarding her performance, appearance, and patterns of negative thinking. We replaced her negative schemas, born from a difficult childhood and unpredictable upbringing, with realistic schemas based on her successes and hard work, and encouraged goal-setting. I also recommended meditation and mindfulness exercises to settle into the discomfort of accepting uncertainty, reduce risk for future depressive episodes, and reduce the chronic stress that contributed to her somatic symptoms. Her physician encouraged a daily regimen of exercise to relieve some of the symptoms of depression and encourage better physical self-care. A few sessions of psychoeducation on sleep hygiene and alcohol and substance abuse also provided her the opportunity to make more informed choices regarding self-medication

and sleep habits. I felt all of these approaches would not only address current symptoms but reduce the potential for experiencing another Major Depressive Episode and increase in clinically significant OCD symptoms.

Fortunately, by facilitating a level of accurate “reality testing”, I was able to help her alleviate some depressive and anxious symptoms. Additionally, the mindfulness exercises, combined with medication and ERP, reduced the frequency of obsessive thoughts and the desire to respond with a compulsive mental review or avoidance. She was also able to make peace with her father’s death and take small steps to widen her social circle without the influence of alcohol. The frequency of her somatic symptoms were reduced, including a reduction in blood pressure, and exercise had not only improved her symptoms of depression, but improved physical confidence. Combined with skills learned in IPT, she felt ready to seek healthier relationships and invest in their maintenance. Given the rate of relapse, I recommended she continue the clomipramine and periodic ERP therapy, as well as maintain habits acquired during CBT and relaxation training.